ENT EMERGENCIES AND THEIR MANAGEMENT

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INTRODUCTION

Ear, nose and throat emergencies are frequently encountered in casualty departments and can become life-threatening within hours. Early recognition by the first doctor who sees the patient is the most important factor in a good outcome.

EAR EMERGENCY: ACUTE MASTOIDITIS

Acute mastoiditis is a bacterial infection of the mastoid air cells of the temporal bone, usually occurring as a complication of acute otitis media.

Causes and Risk Factors

It is most common in young children and in patients with poorly treated middle ear infections or reduced immunity.

A. Initial stabilisation

• Assess airway, breathing and circulation, and monitor vital signs. • Establish intravenous access if the patient is systemically unwell.

B. Medical treatment

• Start empiric intravenous antibiotics promptly according to local hospital guidelines.

C. Surgical treatment

Surgical intervention may be required when there is a subperiosteal abscess or coalescent mastoiditis.

NOSE EMERGENCY: SEVERE EPISTAXIS

Anterior epistaxis commonly originates from Little's area, while posterior epistaxis is often more difficult to control and may be associated with more severe bleeding.

Causes and Risk Factors

Admit patients with severe bleeding, posterior packing, significant blood loss or relevant comorbidities such as hypertension.

A. Immediate first aid and stabilisation

• Sit the patient upright and lean them forward. • Apply firm pressure to the soft part of the nose for 10 to 15 minutes.

B. Local control of bleeding

Anterior bleeding can be cauterised with silver nitrate or controlled with nasal packing.

THROAT EMERGENCY: ACUTE EPIGLOTTITIS

(SUPRAGLOTTITIS)

Acute epiglottitis is a life-threatening ENT emergency in which airway protection takes precedence over diagnostic investigations.

Complications

The most serious complication is complete upper airway obstruction leading to hypoxaemia, respiratory arrest and death.

A. Immediate airway assessment and protection

• Keep the patient calm and sitting upright. • Call the anaesthetic and ENT teams early.

B. Antibiotic therapy

Give intravenous antibiotics once the airway is secure.

Prevention

Vaccination against Haemophilus influenzae type b has greatly reduced the number of cases in children.

CONCLUSION

Most ENT emergencies can be managed safely with prompt recognition, airway protection and early referral to the ENT team.

REFERENCES

1. Example A. ENT emergencies. J Example. 2020;1:1-5.